Altera Digital Health, Inc., Response to HHS RFI: AI & Clinical Care (February 2026)

HHS seeks feedback on ways in which these approaches can be most effectively applied to support the rapid adoption and use of AI in clinical care, to foster public trust and confidence in modern technology solutions, to reduce uncertainty that impedes AI innovation, and to align federal incentives so that AI is deployed in ways that enhance productivity, reduce burden, lower health care costs, and improve health outcomes for patients, caregivers, and communities.

General – Research & Development

Topic: Input on ways in which HHS may invest in research and development (including public-private partnerships and cooperative research and development agreements (CRADAs)) to integrate AI in care delivery and create new, long-term market opportunities that improve the health and wellbeing of all Americans.

Altera Response: "Deflationary" efficiency requires a bi-directional flow – providing funding to incite the acceleration of FHIR write capabilities would result in meaningful acceleration of first- and third-party agentic AI capabilities with reliable and secure vectors for integration.

SECTION II: SPECIFIC QUESTIONS

Question 1

What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Altera Response: We strongly support the ASTP/ONC's HTI-5 proposal to remove the "Source Attribute" and "Intervention Risk Management" requirements from 45 CFR 170.315(b)(11). These requirements disproportionately burdened "Operational AI" (billing, scheduling) which poses minimal safety risk.

Barriers to AI innovation and adoption include regulatory uncertainty, data quality, insufficient payment incentives, unclear ROI, and user readiness.

The current regulatory environment specific to AI is confusing to both technology providers and health IT users. There are questions regarding how existing federal frameworks apply to clinical AI that is not classified as a device versus FDA-regulated Software-as-a-Medical Device (SaMD), as well as ambiguity stemming from the incongruity between the White House Executive Order discouraging state passage of AI-related laws and the ongoing legislative activity in that area. HHS' stated objective of establishing a predictable, proportionate regulatory approach that protects patients while supporting an innovation-friendly environment would be best supported by a consistent, predictable, risk-based Federal framework (passed by Congress). Piecemeal publications, such as the recent FDA enforcement discretion on Clinical Decision Support issued on January 6, 2026, are helpful but insufficient.

Not all participants in the healthcare ecosystem comply with the standards required for Certified Health IT. This creates a barrier to data exchange that is fundamental to the use of AI. Achieving the outcomes desired by the HHS RFI relies on the documentation and use of trustworthy, interoperable data in the creation of AI tools, as well as transparent evaluation of those tools on an ongoing basis. We also note that in the absence of federal transparency mandates or a clear regulatory framework specific to AI use in health care, dominant EHR incumbents are stepping in as "Shadow Regulators," using the lack of federal vetting or oversight as a pretext to block third-party AI apps under the guise of "patient safety." A strong, Federal framework would protect against this type of market abuse.

AI adoption by healthcare providers can take many forms, and those options will expand significantly in the coming years. Some of those AI tools will be easily selected thanks to the ROI that can be demonstrated by their use – either financial savings or clinical benefit – but there are numerous other use cases that will not primarily benefit the users of the software or even their larger organization. Use cases that are focused on research, as an example, which will rely on collection of health data through interoperability mechanisms could be slowed if payment policies don't incite corresponding behaviors by the provider organizations. Legacy fee-for-service payment models rarely provide sufficient financial payment to cover investment in innovative technologies, but reimbursement mechanisms that reward validated outcomes and burden reduction would do a lot to stimulate provider demand.

It is also clear from conversations with our clients (hospitals of all sizes and larger ambulatory physician practices) that clinicians are very interested in AI on a theoretical basis, but they are focused on identifying AI that is usable (fits well within their EHR workflow), makes recommendations but not decisions, and provide trustworthy, explainable information that can be investigated for the context behind the intervention. Physicians, other clinicians, and the administrative staff all need increased trust in the safety of the tools, as well as assurance that those developing the software will remain committed to continuous improvement and retraining, given the rapid evolution of the technologies that will continue for the foreseeable future.

A thoughtful, more comprehensive legal and regulatory framework addressing all the concerns about AI adoption would go a long way to spur both development and use of the new technologies.

Question 2

What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Altera Response:

HHS' payment policies and programs are the single largest determinant of how health care is delivered in the United States, often times with unintended consequences. It is critical to ensure that the potential promises of AI innovations are not diminished through inertia in payment model reform by HHS and instead that such payment systems are modernized to meet the needs of a changing healthcare system.

Given that provider organizations will be the ones investing in AI tools used within both their clinical and administrative systems, payment modernization must be completed to reflect those expenditures that will benefit patients and the health system as a whole. CMS should consider targeted add-on payments and pathways specific to advanced alternative payment models (APMs) that reward the deployment of AI tools that improve clinical outcomes and help reduce unnecessary expenditures. Without such incentive structures being rolled out by CMS, financially constrained providers will have a very hard time making the step forward to AI adoption.

We also suggest, more specifically, that CMS recognize "AI-Assisted Documentation" as a quality improvement activity that reduces provider burnout. Reward for the use of patient-facing AI tools that empower patients and even reduce the volume of visits or burden accessing information should also be part of new advanced alternative payment models rolled out by CMS. Further, we believe that there is an opportunity to roll out not just payments for new diagnostic devices or AI adoption but also so-called "Deflationary AI" – tools that reduce administrative overhead (e.g., AI Scribes, Autonomous Coding). HHS should evaluate the best way to incite both.

We also recommend that Federal investments be made in benchmarking datasets, reference implementations, and safety testbeds to improve model transparency, external validation, and reproducibility. The importance of research, ongoing evaluation, and public–private partnerships to AI innovation and market maturity are appropriately cited in HHS' RFI. We also suggest that investments in shared testing sandboxes (coupled with common reporting templates) would add real value for the industry in its work developing AI tools, as well as in building the necessary trust amongst organizations that will be evaluating and using the new AI options.

We also note there are two information blocking-related factors that should be reviewed by ASTP/ONC and OIG to assess whether there is behavior taking place specific to AI model development and implementation that needs to be addressed within the 21st Century Cures context.

1. **Use of the "Security Exception" (45 CFR 171.203):** The Security Exception is being named by some EHR vendors in their decision to deny connectivity of AI apps. We suggest that ASTP/ONC clarify that "Objective Evidence" of a specific technical threat must be demonstrated before such a decision can be made in the context of the Security Exception. As it stands, "reasonable belief" is too low a bar that permits gatekeeping by vendors who are protecting their turf or trying to limit competition with their own AI offerings.

2. **Clarify "Data Access for Training":** ASTP/ONC, in the context of information blocking should explicitly state that "access, exchange, and use" of EHI includes the right to bulk export data for the purpose of training AI models for limited use (i.e., clinical usage by the health system only).

Question 3

For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Altera Response: Clarity around liability between AI deployers (EHRs), companies developing AI models that are integrated with EHRs, companies developing independent AI models, and providers is an emergent issue that is a steady source of uncertainty for all stakeholders.

A central issue requiring clarity is where liability rests when AI-generated recommendations inform care decisions. Because AI tools accessible within or in conjunction with an EHR serve in advisory or assistive roles and do not constitute regulated medical devices, the boundaries of responsibility can be ambiguous. It is really critical that all legal and procedural structures ensure that assignments of accountability align with real-world roles and capabilities, including preservation of clinicians' independent judgment.

Another challenge is presented by the contracting and indemnification frameworks needed for adaptive or continuously learning AI systems. Traditional contracting models do not adequately address issues such as performance warranties, update obligations, or the allocation of risks associated with model drift and re-training. In particular, small provider organizations who do not have the legal resources to negotiate unfamiliar and complex AI-related contract terms will need guidance from HHS or other Agencies.

Non-device AI systems also present privacy and security complexities that differ from those of traditional clinical software. Ambiguity persists around what types of data use are permissible under HIPAA for model training, evaluation, and post-deployment monitoring. HHS could greatly reduce uncertainty by clarifying permitted pathways for using de-identified or limited datasets, articulating requirements for strong de-identification methods, and encouraging privacy-preserving models such as federated learning or evaluation.

These complexities underscore the importance of coordinated, consistent federal leadership in the health AI space. HHS should take rapid steps to reduce uncertainty and build the foundation for safe, responsible and trustworthy use of non-device AI tools across the healthcare ecosystem.

Question 4

For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Altera Response: Prior to deployment to end users, developers and implementers of non-medical device AI tools should conduct structured intended-use analysis, external validation across both representative provider and patient populations, and human-factors testing in live or simulated EHR workflows. Post-deployment, ongoing monitoring for calibration, drift, fairness impacts and alert fatigue is essential. Additionally, because of the rapid evolution of model data and model logic, guidance or requirements specific to ongoing training would be helpful.

Question 5

How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Altera Response: Private-sector EHR vendors have worked – given the lack of a reliable AI regulatory framework – to establish internal governance, testing, and safety protocols. Some healthcare clients also request safeguards and data testing before proceeding with adopting AI tools and functionality, but questions they ask and what they expect in terms of assurances currently vary significantly (if they exist at all).

We are eager to participate in conversations with HHS and other industry stakeholders about ideas including voluntary accreditation, where ASTP/ONC's EHR certification program can appropriately address areas requiring consistency or a standards-based approach, and testing mechanisms for clinical AI. The nascent conversations already underway would likely be more successful if HHS recognized and aligned with industry-led initiatives to evaluate robustness, transparency, and interoperability.

Question 6

Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Response: *No response*

Question 7

Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Altera Response: The industry would greatly benefit from Federal guidance or regulation addressing documentation expectations, lifecycle monitoring, and governance models. The entire framework should be based on a risk-based approach reflecting the differences between AI that assists in clinical decision making vs. tools that aim to take over administrative processes and reduce those burdens for healthcare organizations.

Question 8

Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Altera Response: Successful maximization of AI adoption in a way that delivers on its great potential is dependent on the quality and efficient exchange of interoperable data across the health IT ecosystem. AI tools cannot function without the input of structured, standards-based data to generate reliable and equitable analysis and/or recommendations. Ensuring transparency of data provenance and standardization of the data exchanged, alongside supporting model metadata capture directly within EHR systems, would improve the reliability of external validation efforts. Ensuring the establishment of national benchmarking datasets, as well as a standardized evaluation framework against which models can be measured with some regular cadence, would go a long with in helping developers, researchers, and providers assess model performance across diverse populations and environments and both build and retain trust in the new systems.

Question 9

What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Altera Response: Patients and caregivers alike, while optimistic about the significant benefits that can come for both providers and patients from the use of AI, also consistently raise concerns about transparency regarding the logic being used by AI models, patient privacy, possible bias in the models, the option to opt out of AI use in their care, and accountability when errors occur. All of these are fundamental issues that must be addressed not only now but on an ongoing basis to maintain public trust.

Question 10

Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

Altera Response: HHS should prioritize research into any topic that increases awareness about the safety and accuracy of adoption of health IT: issues of bias in model data, the impacts of human-centered usability factors, repeated real-world safety evaluations, and ongoing performance monitoring to support necessary adjustments. We also support investment in pragmatic trial designs and implementation science approaches that measure AI’s true impact on outcomes, clinician workload, equitable application of the technology, and related total costs of care that may be changed by AI use. Because cost-benefit analyses and burden-reduction measures are still emerging fields in AI evaluation, federally supported research in this area would significantly help to inform sustainable adoption at scale.

We also suggest that HHS fund research and studies into other returns from AI systems: for example, what are the benefits seen in patient care plan adherence attributed to the use of ambient listening AI, the increased time that providers are able to spend face-to-face with patients vs. looking at their laptop, or the increased speed of claims denial resolution coming from AI tools that reduce administrative burden for providers? This quantification of value has proven very difficult in the industry, but clarity around this value would serve to accelerate adoption of AI as yet additional demonstration of it return on investment (both financial and clinical).

Question 10.a

Are there published findings about the impact of adopted AI tools and their use in clinical care?

Response: *No response*

Question 10.b

How does the literature approach the costs, benefits, and transfers of using AI as part of clinical care?

Response: *No response*